



Transfusion Policy Manual

Refusal / Restricted Consent of Blood and Blood Products

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Clinical staff

Introduction

Competent adults have the legal right of autonomy or self-determination¹, and therefore the legal right to decline medical treatment, including the transfusion of blood and blood products. Declining consent may be based on personal beliefs or experiences, most commonly Jehovah's Witnesses (JW) who have specific beliefs regarding blood and transfusion. Acceptance of products, especially plasma derivatives, may vary according to an individuals' personal conscience.

SCGOPHCG are committed to respecting patients' rights without prejudice, and ensuring best care is provided by partnering with the patient in planning their care.

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☐
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Policy Principles

This policy outlines the requirements for health professionals when a patient declines the use of blood products, in accordance with the [Guardian and Administration Act 1990](#), the [WA Health Consent to Treatment Policy](#) and National Safety and Quality in Health Standard 2.

Clinical staff are responsible for ensuring that patients' right to refuse blood and blood products is heard, documented, and upheld without prejudice, and alternative options discussed as appropriate.

Competent patients will be required to complete a [Refusal / Restricted Consent of Blood Products Form D](#).

The [Refusal of Blood/Blood Products flow chart](#) is available at appendix 1.

The [Clinician Resource for Transfusion Refusal](#) is available at appendix 2.

A patients' "Refusal / Restricted Consent to Blood Products" MUST be communicated at patient handovers and at procedural/operating theatre timeouts.

A patient who does not wish to accept blood or blood products as part of their treatment plan, and who's choice is unwavering, should have an Advanced Health Directive (AHD). Patients should be advised to upload this to My Health Record and a copy of it should be placed in the patients' medical record.

Healthcare professionals should provide AHD information and resources to patients who have identified limitations on healthcare treatments that they are willing to receive.



NOTE: the JW AHD is titled *Advance Health Care Directive: NO BLOOD*. While an AHD may inform patients' Goals of Care should an adverse/catastrophic life event occur, it may also guide an individuals' treatment preferences at any time health treatment is required and is not an "end of life" specific document.

General Principles

- A fully informed, competent adult is entitled to accept or reject medical treatment
- Patients must be provided with information to enable them to make an informed decision regarding blood transfusion and have an opportunity to ask relevant questions
- Exercising self-determination and rejecting specific medical treatments may have consequences upon the nature and extent of treatment options that may be available.
- Jehovah's Witnesses (JW) generally DO NOT accept whole blood or major blood components; however, the acceptance of plasma derivatives **varies** according to the individual. **Clearly establish the patient's current directive on each hospital admission.**
- Many JW carry a signed Advanced Health Directive *No Blood* card which should be consulted to view the patient's wishes regarding treatment options. The card contains the name of an alternative contact/substitute enduring guardian.
- The SCGOPHCG JW liaison can be contacted via switchboard for patient assistance and can provide advice in locating medical staff at other facilities to consult on alternative care.

Note: If medical or nursing staff have any difficulties or queries regarding the treatment or course of action, they should contact the Hospital Executive Director of Clinical Services or delegate for further advice.

Procedure

Where a patient declines the use of blood/ blood products it should be documented/identified as such on the iCM nursing handover and the following forms:

1. **Refusal / Restricted Consent of Blood Products Form D:** MUST be completed
2. **Consent to Treatment Form A:** where "No" has been indicated at the consent to blood product query- this response should be highlighted
3. **Perioperative Record:** in "Other Stickers" section, "Patient has Refusal / Restricted Consent of Blood Products form" should be written and highlighted. A note should be made of any ACCEPTED products e.g., Albumin.
4. **Anaesthesia Record:** at "Anaesthesia Perioperative Plan/Consent"- "Patient has a Refusal / Restricted Consent of Blood Products form" should be written and highlighted. A note should be made of any ACCEPTED products e.g., Albumin.



5. **Area specific Procedure Checklists:** "Patient has Refusal / Restricted Consent of Blood Products form" should be written and highlighted on the first/front page of the checklist in an appropriate alerts or comments section. A note should be made of any ACCEPTED products e.g., Albumin.

1. Management of COMPETENT/CONCIOUS patients refusing blood and blood products

- 1.1 Identify the patient's directive regarding acceptance of receiving blood/blood products- copy of AHD should be filed in the medical record (AHD should be advised if not already in place)
- 1.2 Complete the Refusal / Restricted Consent of Blood Products (Form D) with the patient- patient and medical officer to complete sections as indicated
- 1.3 Review non-blood medical alternatives and determine treatment plan with patient.
- 1.4 For patients undergoing procedures with a high risk of blood loss, consult Haematology for advice. Arrange blood testing per [PBM guidelines](#):
 - a. FBC, Iron Studies, U&Es, LFTs, CRP, Vitamin B12, and folate.
 - b. All requests for the use of erythropoietin must be discussed with Haematology.
- 1.5 If patient is a JW, the SCGOPHCG JW liaison can be contacted via switch to assist in communication and understanding alternative treatment options.
- 1.6 **IN AN EMERGENCY:**
 - a. An adult patient who is **conscious and able to write** should complete the '[Refusal / Restricted Consent of Blood Products \(Form D\)](#)'. It is medical staff's responsibility to explain this form to ensure legal validity.
 - b. If the patient is conscious and competent, but unable to write/or emergency precludes the formality of Form D completion, and a consent discussion has occurred in the emergency setting- verbal refusal of blood products obtained by a medical officer with another medical officer as a witness, should be documented in the patients' medical record with names and signatures of both.

2. Management of an UNCONCIOUS patient or a PATIENT DEEMED NOT COMPETENT who it is believed would refuse blood and blood products

- 2.1 If an unconscious/not competent adult patient had previously completed an AHD, it should be sought to guide treatment decisions. Upon confirmation of patient ID, clinical staff should check the patients My Health record, and/or wallet/purse for a current AHD. The AHD must comply with the requirements of the Guardianship and Administration Act 1990.
- 2.2 The AHD should clearly state the patients' choices on the use of blood products, including which treatments they consent to, and which treatments they refuse.
- 2.3 The AHD should be copied and filed into the patients' medical record and a medical officer should also document the same, including the patients accepted transfusion treatment options.
- 2.4 If an AHD is not located, patient ID has been confirmed, and it is firmly believed that the patient would decline the use of blood/blood products, attempts to confirm patients' treatment decisions should be sought. Attempts should be made to contact patients next of kin, enduring guardian, GP, Church Minister or the JW Hospital Liaison.
- 2.5 If attempts to contact are futile, attempts should be made to locate a substitute decision maker as defined by the [Guardianship and Administration Act 1990](#) Section 110 ZJ: *Order of priority of persons who may make treatment decision in relation to patient (p.101)* and 110 ZD: *Circumstances in which person responsible may make treatment decision (p. 95)*.



- 2.6 If the substitute decision maker has not been given the Enduring Power of Guardianship or is the Guardian appointed by the State Administrative Tribunal (SAT), an application to the SAT should be considered.
- 2.7 **IN AN EMERGENCY:** If unable to gain transfusion refusal information, treat as clinically necessary in accordance with the *Division 2- Provision of treatment, section 110ZI* “Urgent treatment generally” provisions set out in the [Guardianship and Administration Act 1990](#) (page 100) which states:

“110ZI. Urgent treatment generally

(1) Subsection (2) applies if —

- (a) a patient needs urgent treatment; and
- (b) the patient is unable to make reasonable judgments in respect of the treatment; and
- (c) it is not practicable for the health professional who proposes to provide the treatment to determine whether or not the patient has made an advance health directive containing a treatment decision that is inconsistent with providing the treatment; and
- (d) it is not practicable for the health professional to obtain a treatment decision in respect of the treatment from the patient’s guardian or enduring guardian or the person responsible for the patient under section 110ZD.

(2) The health professional may provide the treatment to the patient in the absence of a treatment decision in relation to the patient.”²

And, in accordance with the [WA Health Consent to Treatment policy](#):

Treatment without consent must be:

- reasonably required to meet the emergency
- in the patient’s best interests
- the least restrictive of the patient’s future choices.

The rationale for treatment without consent must be clearly documented in the patient’s medical record. The medical record must state details of attempts made to contact the substitute decision maker.”¹

NOTE: All treatment decisions must be documented in the patient’s medical record and on *Refusal /Restricted Consent of Blood Products (Form D)* in conjunction with a treatment plan that has been agreed upon by two medical doctors, where:

- a. blood and/or blood products have been administered without consent, or
- b. where refusal of blood and/or blood products has been directed by an AHD, the Next of Kin, the GP, the Enduring Guardian or a substitute decision maker for an unconscious patient or in a patient who has been deemed not competent to consent.

3. Guidelines for the EMERGENCY treatment of patients under 18 years whose parents refuse blood and blood products

The Human Tissue and Transplant Act 1982 addresses the removal of blood and the administration of blood and/or blood product transfusion to a child in the absence of parental consent.

Human Tissue and Transplant Act 1982 Part II Division 5 Section 21 (subsection 1-2) states:

- 3.1 A medical practitioner may perform a blood transfusion upon a child without the consent of any person who is legally entitled to authorise the blood transfusion if



- a) such person
 - i. fails or refuses to so authorise the blood transfusion when requested to do so; or
 - ii. cannot be found after such search and enquiry as is reasonably practicable in the circumstances of the case; **and**
- b) the medical practitioner and another medical practitioner agree
 - i. as to the condition from which the child is suffering; and
 - ii. that the blood transfusion is a reasonable and proper treatment for that condition;
 - iii. that without a blood transfusion the child is likely to die; **and**
- c) the medical practitioner who performs the blood transfusion on the child
 - i. has had previous experience in performing blood transfusions; and
 - ii. has, before commencing the transfusion, assured himself that the blood to be transfused is suitable for the child.

3.2 When a medical practitioner has performed a blood transfusion on a child without the consent of any person legally entitled to authorise it and in respect of that transfusion the requirements and conditions of this section have been complied with, the transfusion shall be deemed for all purposes to have been performed with the authority of a person legally entitled to authorise it.

Human Tissue and Transplant Act 1982 Part II Division 5 Section 21 (subsection 3) states:

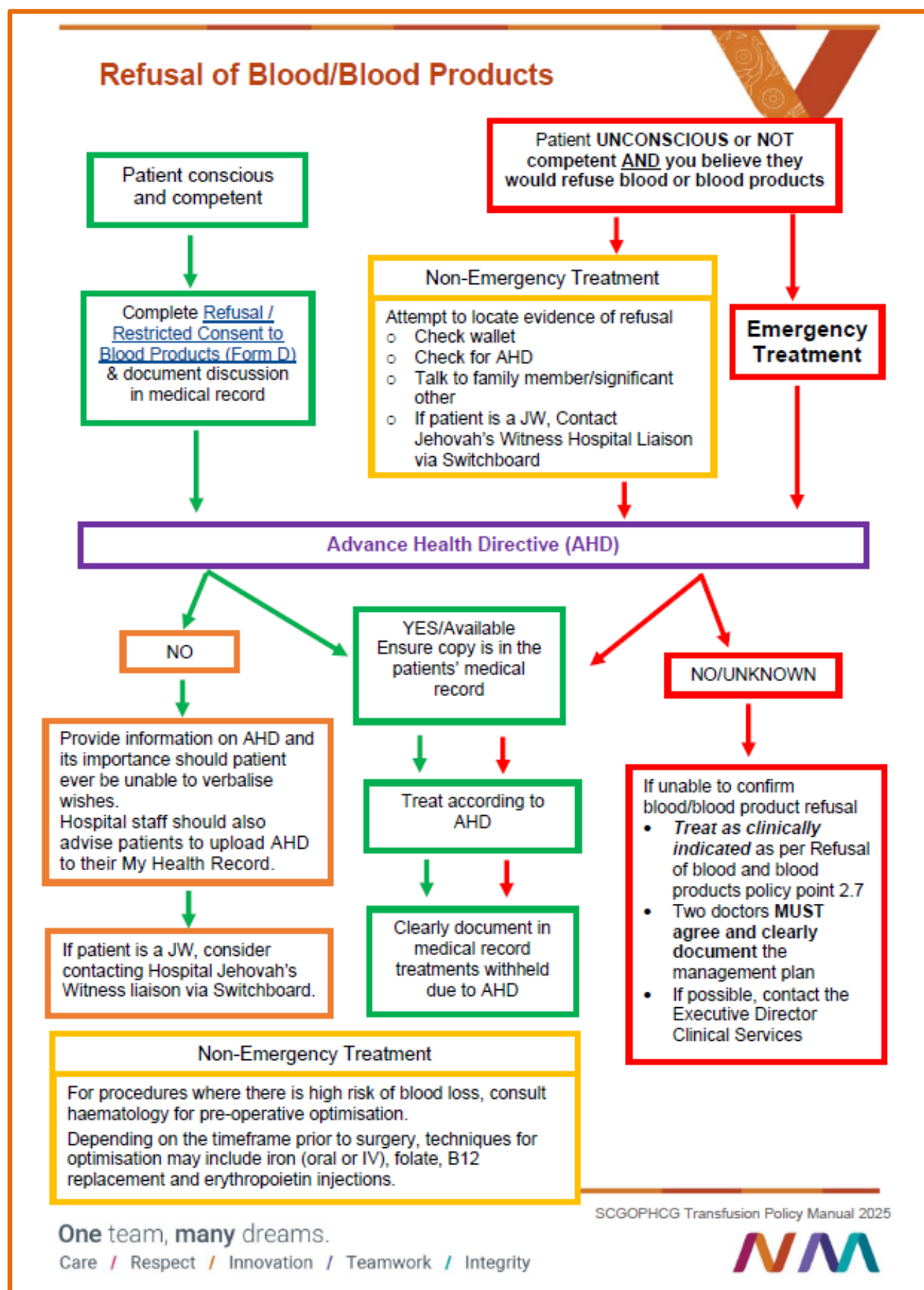
- 3.3 Where a medical practitioner other than the medical practitioner who is to perform the blood transfusion on the child cannot be found after search or enquiry for such time as the last-mentioned medical practitioner considers reasonable in the circumstances of the case, having regard to the emergency arising from the condition of the child, it is sufficient compliance with subsection (1-b) if that last-mentioned practitioner satisfies himself
- a) as to the condition from which the child is suffering; and
 - b) that a blood transfusion is a reasonable and proper treatment for that condition; **and**
 that to delay the blood transfusion until that other medical practitioner can be found and be available for consultation would cause a serious deterioration in the child's condition; **and**
 - c) that without a blood transfusion the child is likely to die.

Human Tissue and Transplant Act 1982 Part II Division 5 Section 21 (subsection 4-5) states:

- 3.4 In this section **blood transfusion** means the transfusion of human blood, any constituent of human blood, into a child and includes the exchange of the whole or any part of the blood of a child and all medical and surgical procedures necessary to perform the transfusion or exchange; and **child** means a person who is or appears to be under the age of 18 years.
- 3.5 Nothing in this section relieves a medical practitioner from liability in respect of the administration of a blood transfusion to a child being a liability to which he would have been subject if the transfusion had been administered with the consent of a parent of or a person having authority to consent to the administration of the transfusion.



Appendix 1



Appendix 2

Clinician resource for transfusion refusal (consent form D): Identification of blood product source	
Description	Indication
Fresh Blood Components (derived from whole blood)	
Red Blood Cells	Blood loss. Symptomatic anaemia. Bone marrow failure.
Platelets	Thrombocytopenia. Massive blood loss. Platelet function abnormalities.
Fresh Frozen Plasma	Replace missing or low levels of proteins due to some medical conditions or following severe blood loss.
Products derived from Plasma (fractions)	
Cryoprecipitate	To treat low levels of clotting factors.
Albumin	To treat low albumin levels. Can be used for low blood volume or priming lines in specialised procedures.
Immunoglobulin	Contains antibodies which help the immune system to function properly. Under production, auto immune conditions.
Factor concentrates from Plasma (fractions)	
Prothrombin Complex Concentrate (Beriplex®)	Reversal of Warfarin. Rx of specific clotting factor deficiency.
Fibrinogen Concentrate	Haematologist prescribed. Rx of acute bleeding in pts with congenital fibrinogen deficiency (prophylaxis in high-risk pts). Massive Transfusion protocol.
MonoFIX-VF	Haematologist prescribed: Haemophilia management.
Biostate	Haematologist prescribed: Rx for von Willebrand disease and sometimes factor VIII deficiency due to Haemophilia A.
Other plasma derived products (fractions)	
Fibrin glues	Formulation from fibrinogen (human) & thrombin (bovine) applied to tissue sites to glue them together.
Thrombotrol-VF (antithrombin III)	Haematologist prescribed: Pts with inherited deficiency of antithrombin: prophylaxis and Rx thrombosis and thromboembolism during surgery, pregnancy, and childbirth. May be used in acquired anti-thrombin deficiencies e.g., liver disease and acute DIC.
Recombinant clotting factors (no human serum or proteins)	
RFVIIa (NovoSeven)	Haematologist prescribed: Haemostasis management of Haemophilia A or B with inhibitors, congenital Factor VII deficiency and Glanzmann's thrombasthenia with refractoriness to platelet transfusions. Sometimes used in Massive Transfusion.
Xyntha, Advate, BeneFIX	Haematologist prescribed: Haemophilia management.
Others	
Fibrin glues and sealants (plant or animal)	Plant/animal-based products applied to bleeding surfaces to help control local bleeding or seal wounds during surgery. See specific product information.
Procedures	
Acute normovolaemic haemodilution	Depends on pt, surgery type and Anaesthetist preference: Involves removal of ~400-500mls of the pt's blood prior to/shortly after anaesthesia induction. 'Blood loss' is replaced with IV fluid, diluting the remaining blood, blood volume relatively unchanged. On completion of the surgery, the patient's own blood is re-infused.
Cell salvage	Intra-operative collection of the pt's own blood cells which are added to anticoagulant, then filtered and washed prior to reinfusion to the same pt.

V1 Dec 2016, V2. April 2020, V 3 July 2021, V4 July 2022, V5 March 2023, V6 November 2025

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HUMAN derived products	
Albumin:	Human albumin solution. Manufactured from human plasma collected in Australia. PI Updated 01/03/2023
Beriplex®:	sterile freeze-dried powder containing purified human coagulation factors II, VII, IX and X. Manufactured from human plasma collected in Australia. PI updated: 28/06/2024
RiaSTAP® (Fibrinogen Concentrate):	sterile, freeze-dried fibrinogen (coagulation factor) concentrate derived from human plasma. PI updated: 01/07/2020
Monofix®- VF:	sterile freeze-dried powder containing purified human coagulation factor IX. Manufactured from human plasma collected in Australia PI updated: 01/06/2020
Biostat®:	sterile powder containing a human coagulation factor VIII (FVIII) and human Von Willebrand factor (VWF) complex purified from cryoprecipitate. Manufactured from human plasma collected in Australia. PI updated: 01/11/2021
Thrombotrol®-VF:	sterile, freeze-dried preparation of purified human antithrombin III (ATIII). Manufactured from human plasma collected in Australia. PI updated: 01/05/2020
NON human derived products	
NovoSeven® RT:	contains activated recombinant coagulation Factor VII of approximate molecular mass 50K Daltons produced by genetic engineering from baby hamster kidney cells (BHK cells). The recombinant coagulation Factor VIIa (rFVIIa) in NovoSeven® RT is structurally very similar to human plasma-derived activated Factor VIIa. PI updated: 01/12/2018
Xyntha®:	Moroctocog alfa, also known as recombinant coagulation factor VIII. Moroctocog alfa (rch) is a purified protein produced by recombinant DNA technology for use in factor VIII deficiency. It is secreted by a genetically engineered Chinese hamster ovary (cho) cell line. The cell line is grown in a chemically defined cell culture medium and therefore does not contain any materials derived from human or animal sources. PI updated: 01/03/2023
Advate®:	Octocog alfa, recombinant antihaemophilic factor VIII Plasma/Albumin-Free Method (rAHF-PFM), is produced from a genetically engineered Chinese hamster ovary (CHO) cell line and manufactured by a method that is free from the use of animal or human derived proteins. PI updated: 01/12/2020
BeneFIX®:	Nonacog alfa, produced by recombinant DNA technology for use in Factor IX deficiency. Produced by a genetically engineered Chinese hamster ovary (cho) cell line that does not contain any proteins derived from animal or human sources. BeneFIX is inherently free from the risk of transmission of human blood borne pathogens such as HIV, hepatitis viruses and parvovirus. PI updated: 01/09/2019

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Compliance, monitoring and evaluation

The SCGOPHCG Blood Management Committee is responsible for ensuring the effectiveness of this policy and will monitor compliance through clinical incident review and consumer feedback.

Related Documents

Legislation:

- [Guardian and Administration Act 1990](#)
- [WA Health Consent to Treatment Policy](#)
- [Human Tissue and Transplantation Act 1982](#)

Authorising Policy and Standard/s:

- NSQHS Standard 2 Partnering with Consumers
- NSQHS Standard 7 Blood Management

Standards, procedures, guidelines:

- [ANZSBT Guidelines for Administration of Blood Products](#)

Supporting documents

- [Refusal / Restricted Consent of Blood Products \(Form D\)](#)

References

1. Government of Western Australia Department of Health. Consent to treatment policy (Internet). Perth: Government of Western Australia; 2023 Mar (cited 2023 Mar 14). Available from: <https://www.health.wa.gov.au/About-us/Policy-frameworks/Clinical-Governance-Safety-and-Quality/Mandatory-requirements/Consent-to-Treatment-Policy>
2. Government of Western Australia. Guardianship and Administration Act 1990 (Internet). Perth: Government of Western Australia; 2022 Jul (cited 2023 Mar 30). Available from https://www.legislation.wa.gov.au/legislation/statutes.nsf/main_mrtitle_406_homepage.html
3. Australian and New Zealand Society of Blood Transfusion, Australian College of Nursing. Guidelines for administration of blood products (Internet). Sydney: Australian & New Zealand Society of Blood Transfusion Ltd; 2019 Oct (cited 2023 Mar 21). Available from: <https://anzsbt.org.au/guidelines/guidelines-for-the-administration-of-blood-products/>

Authorisation & Version Control

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Applicable Standards	NSQHS Standards: 				
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1.0 Document Control

[This section will be completed by SCGOPHCG Policy Unit]

Version	Policy author	Summary of changes	Approval date

